

August 12, 2026

Absolute Total Care (Centene)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Stanford Windler (MRN MUSC9546840), Claim MUSC-20260520-1B1C-1

Patient	Stanford Windler (DOB 1941-08-06)
MRN	MUSC9546840
Member ID / Group	ABS330201565 / GRP-63437
Claim number	MUSC-20260520-1B1C-1
Date of service	2026-05-20
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$554.11
Denial code	CARC 197 / RARC N54 — Precertification/authorization/notification/pre-treatment absent.
Appeal deadline	2026-10-11

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health University Medical Center is submitting this first-level appeal on behalf of patient Stanford Windler (MRN: MUSC2704695, Member ID: ABS330201565) regarding the denial of Claim MUSC-20260520-1B1C-1 for a moderate-complexity established-patient office visit (CPT 99214) rendered on May 20, 2026, by James H. Ferrell, MD (NPI 1992837465). The claim was denied by Absolute Total Care (Centene) on July 13, 2026, under CARC 197 and RARC N54, with the payer remark stating: "No prior authorization on file for the date of service billed." The total denied amount is \$554.11. MUSC Health respectfully contends that this denial is inconsistent with applicable Medicaid managed care requirements governing prior authorization for routine outpatient evaluation and management services, and requests full reversal and payment at the allowed rate of \$319.26.

CLINICAL BACKGROUND

Mr. Windler is an 85-year-old male with an extensive and well-documented chronic disease burden managed longitudinally at MUSC Health. His active conditions include coronary heart disease (I25.10), for which he carries prescriptions for simvastatin, amlodipine, nitroglycerin, and clopidogrel — all initiated in May 2014 and currently active. He also carries active diagnoses of osteoporosis, gout, anemia, prediabetes, and obesity, as well as a longstanding history of chronic sinusitis dating to 1952. The May 20, 2026 encounter was billed under primary diagnosis J06.9 (acute upper respiratory infection, unspecified) with secondary diagnosis I25.10 (coronary heart disease), reflecting the complexity of managing an acute presentation in the context of significant underlying cardiovascular and metabolic comorbidities. Dr. Ferrell is the rendering provider and an established treating physician for this patient within the MUSC Health system.

BASIS FOR APPEAL

The denial under CARC 197 and RARC N54 asserts that no prior authorization was on file for the date of service. MUSC Health disputes this characterization on two grounds.

First, under federal Medicaid managed care regulations and widely recognized standards applicable to Medicaid MCOs, routine outpatient evaluation and management visits — particularly for established patients presenting with acute illness — are generally not subject to prior authorization requirements. CPT code 99214 represents a standard office visit, not a procedure, specialty service, or high-cost intervention that would ordinarily trigger a precertification obligation. If Absolute Total Care's plan documents impose a prior authorization requirement on established-patient E/M visits at this level of complexity, MUSC Health requests that the payer identify the specific contractual or plan provision that mandates such authorization, as this requirement is not consistent with standard Medicaid managed care practice and may not have been communicated to network providers through the standard credentialing and contract notification process.

Second, even if a prior authorization obligation existed, denial under RARC N54 — indicating that claim information is inconsistent with pre-certified services — presupposes that some authorization was sought or issued. The payer's own remark states that no authorization is on file, which is logically inconsistent with the application of N54. This coding discrepancy further undermines the basis for denial and suggests the claim may have been processed in error.

Mr. Windler's active cardiovascular medications, his age, and the complexity of his comorbid conditions fully support the medical necessity of a moderate-complexity evaluation and management visit. The services rendered were appropriate, medically necessary, and consistent with the standard of care for an 85-year-old patient with coronary heart disease presenting with an acute upper respiratory condition.

REQUESTED ACTION

MUSC Health respectfully requests that Absolute Total Care (Centene) reverse the denial of Claim MUSC-20260520-1B1C-1 in its entirety and issue payment at the contracted allowed amount of \$319.26 for CPT 99214 rendered on May 20, 2026. If the payer maintains that prior authorization was required, we further request that Absolute Total Care provide, in writing, the specific plan provision or provider communication establishing that requirement, along with the retroactive authorization process available under applicable Medicaid managed care appeal rights. This appeal is submitted within the 90-day dispute window, ahead of the October 11, 2026 deadline, through the secure provider portal with supporting documentation attached.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record